Outlook

RE: Policy ownership

From Holiday, Charisse (IHS/PHX) <Charisse.Holiday@ihs.gov>
Date Thu 29-Aug-24 19:08
To Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>; Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>; Numkena, Brian G (IHS/PHX) <Brian.Numkena@ihs.gov>
Cc Valdisera, Paula (IHS/PHX/PIMC) <Paula.Valdisera@ihs.gov>; McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>

Hello,

Thank you for the thorough review of the policy. Your review draws out some concerns, more specifically, Brian being designated as the RSO for the hospital. To clarify, Brian is not an expert in radiation safety; his role within the department is the Practice Manager and his training, background, and experience is strictly administrative. Maybe ownership requires further discussion, but Brian did speak with someone at Area who recommended contracting the role out to a physicist.

Happy to discuss further,

Charisse

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Thursday, August 29, 2024 11:12 AM
To: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>; Numkena, Brian G (IHS/PHX) <Brian.Numkena@ihs.gov>
Cc: Valdisera, Paula (IHS/PHX/PIMC) <Paula.Valdisera@ihs.gov>; Holiday, Charisse (IHS/PHX) <Charisse.Holiday@ihs.gov>; McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Subject: Re: Policy ownership

Dear Clint,

Thank you for including me in this critical discussion. I appreciate the opportunity to contribute to ensuring that our radiation safety policies are comprehensive and compliant with all necessary regulations.

After reviewing LT Valdisera's updated draft policy, several key issues and discrepancies have been identified, including conflicts in RSO designation, responsibilities for the Radiation Monitoring Program, and dosimetry badge monitoring. The current and draft policies differ significantly in terms of department roles and responsibilities, which could affect compliance and safety. The RSO's designation must be in writing, meet specific qualifications, and be properly documented to ensure regulatory compliance. A reassessment of these policies is necessary to align with regulatory mandates and maintain safety standards across departments.

I have identified several key issues and discrepancies that I feel need to be addressed to ensure that the policy is comprehensive and compliant with regulatory mandates.

Gaps in Current Policies:

1. **Policy 9897096 (Radiation Safety and Monitoring):**
    - This policy assigns RSO duties to the Safety and Occupational Health Specialist, which conflicts with the draft policy's suggestion that the RSO should be a member of the medical imaging team

or a department supervisor. Additionally, this policy places responsibility for the Radiation Monitoring Program under the Environmental Health and Safety (EHS) department, while the draft policy shifts this responsibility to the Radiology Department. These discrepancies indicate a need to reassess the proper delegation of duties and ensure that the RSO is qualified and appropriately positioned within the organization. Brian is not qualified and not trained to do this. He is a practice manager and is strictly administrative.

2. **Policy 13677097 (Pathology/Laboratory Radiation Safety Policy):**
   - This policy needs comprehensive guidance on radiation safety measures, particularly in defining the qualifications, roles, and responsibilities of the Radiation Safety Officer (RSO), and it needs to adequately address procedures for radiation monitoring and safety training. These gaps could lead to non-compliance and potentially compromise the safety of our personnel.
3. **Policy 15289088 (Dental Radiation Safety Policy):**
   - The policy references the "Phoenix Service Unit Radiation Safety and Monitoring Policy," which currently does not exist or is not accessible. This creates potential compliance issues and may leave staff without guidance to ensure proper radiation safety practices.

Discrepancies and Inconsistencies Identified:

1. **RSO Designation:**
   - The current policy assigns the RSO duties to the Safety and Occupational Health Specialist. In contrast, the draft policy suggests the RSO should typically be a member of the medical imaging team or a department supervisor when not appointed by the CEO. This discrepancy indicates a potential need to reassess the appropriate individual or department for the RSO role. While the policy is not required to outline who the RSO is, there must be an official delegation, and this information must be easily accessible by staff. It is crucial that the agency delegates the RSO in writing and completes the Arizona Department of Health Services ARRA-DOA Delegation of Authority to ensure compliance.
2. **Administration of Radiation Monitoring Program:**
   - The current policy assigns the administration of the Radiation Monitoring Program to the Environmental Health and Safety (EHS) department, while the draft policy assigns this responsibility to the Radiology Department. This change could impact the program's management and accountability.
3. **Dosimetry Badge Monitoring Responsibilities:**
   - Responsibilities for dosimetry badge monitoring shift between the current and draft policies. The current policy places these responsibilities under the EHS Officer, while the draft policy involves the Radiation Safety Officer and the Radiology Department Manager more directly.
4. **Roles of EHS Officers:**
   - The draft policy alters some responsibilities from EHS Officers to department-specific management, particularly in the Radiology Department, which may affect the chain of command and responsibilities for radiation safety.
5. **Training and Competency Requirements:**
   - The current policy for training and competency relies on departmental managers and EHS, but the

draft replaces this with a more collaborative approach involving the Radiology Department and the RSO, potentially affecting how training is monitored.

Required Qualifications for the RSO:

To ensure that the Radiation Safety Officer (RSO) is fully qualified to manage the complexities of radiation safety, the following qualifications should be met, consistent with the guidelines from **10 CFR 34.42**, **10 CFR 35.50**, **NRC**, **VHA Directive 1105**, **AAPM Reports 121 and 160**, and the **Health Physics Society (HPS)**:

- **10 CFR 34.42 (Radiation Safety Officer for Industrial Radiography)**:
  - The RSO must have:
    - **Training and Experience**: Formal training in radiation safety and at least 2 years of practical experience in radiation protection activities related to industrial radiography.
    - **Certification**: Certification as a radiographer by a recognized certifying body is typically required.
    - **Responsibilities**: The RSO is responsible for implementing the radiation protection program, ensuring regulatory compliance, and managing the safety of operations involving the use of radioactive materials and equipment.
- **10 CFR 35.50 (Radiation Safety Officer for Medical Use of Byproduct Material)**:
  - The RSO must have:
    - **Certification**: Certification by a recognized board such as the American Board of Radiology (ABR), American Board of Nuclear Medicine (ABNM), or American Board of Health Physics (ABHP).
    - **Education and Experience**: A degree in physical science, engineering, or a related field with additional training and experience in radiation safety applicable to the medical use of byproduct material.
    - **Responsibilities**: The RSO must ensure that byproduct material is used safely, manage the radiation safety program, and ensure compliance with all applicable NRC regulations.
- **Nuclear Regulatory Commission (NRC) Qualifications**:
  - The RSO must have sufficient training and experience to effectively oversee the radiation safety program. This typically includes:
    - **Education**: A degree in physical science, engineering, or a related field.
    - **Certification**: Certification from boards like the American Board of Radiology (ABR) or American Board of Health Physics (ABHP) may be required depending on the specific license.
    - **Experience**: The RSO must have practical experience with the types of radioactive materials and equipment used within the facility, including knowledge of NRC regulations and radiation protection principles and the ability to implement and maintain a radiation safety program that complies with NRC requirements.
    - **Training**: The RSO should have completed specialized training programs recognized by the NRC, covering radiation safety, regulatory compliance, and emergency procedures.
- **VHA Directive 1105**:

  - **Educational Background**: The RSO should have a degree in health physics, medical physics, radiologic technology, or a closely related field. Advanced degrees (e.g., MS or PhD) are highly preferred.
  - **Experience**: Significant experience in radiation protection, particularly in healthcare settings, including managing radiation safety programs and conducting radiation surveys.
  - **Authority and Reporting Structure**: The RSO should report directly to the facility's executive management, ensuring appropriate oversight and independence in their role.
- **AAPM Reports 121 and 160**:
  - **Technical Knowledge**: Detailed knowledge of radiation physics, radiobiology, and radiation protection instrumentation.
  - **Certification**: Certification by recognized boards such as ABR or ABHP.
  - **Responsibilities**: The RSO is responsible for overseeing the entire radiation safety program, ensuring compliance with ALARA principles, and maintaining safety protocols to protect both patients and staff.
- **Health Physics Society (HPS) Guidelines**:
  - **Educational Background**: A degree in health physics or a closely related field.
  - **Certification**: Strongly recommended certification by ABHP or ABR.
  - **Experience**: Experience in managing radiation safety programs, especially in healthcare settings, is essential.

Benchmarking Against Other Federal Hospitals:

To ensure we are maintaining the highest standards, I have included **VHA Directive 1105** as a benchmark for what other federal hospitals require. This directive outlines comprehensive standards and responsibilities for radiation safety officers within the Veterans Health Administration, providing a valuable reference as we align our policies with best practices.

Additional documents that can be used for benchmarking include:

- **VHA Directive 1129**: This directive provides guidelines for radiation protection for machine sources of ionizing radiation, which complements the requirements of the RSO role in managing radiation safety programs.
- **NUREG-1556, Volume 9**: This NRC document provides detailed guidance on radiation safety for medical use programs, including RSO responsibilities. It serves as an essential reference for ensuring compliance with NRC regulations.
- **Health Physics Society Guidelines**: These guidelines offer a comprehensive overview of RSO qualifications and responsibilities, mainly focusing on healthcare settings.

Required Actions:

- The policy must clearly outline the qualifications required for the RSO, ensuring that the individual meets the educational, certification, experience, and authority criteria outlined above.
- Regulatory compliance is crucial. The **Nuclear Regulatory Commission (NRC)** and **OSHA** standards, as well as the **Arizona Radiation Regulatory Agency (ARRA)**, mandate that facilities designate a qualified RSO who meets the state's standards for radiation safety management. These regulations are

complemented by VHA guidelines for managing radioactive materials and ensuring safety.

- Any policy ownership updates or transfers must ensure that these regulatory standards are met and that the RSO is delegated in writing with the **ARRA-DOA Delegation of Authority** completed.

A formal written designation is crucial because it provides clear documentation of who is responsible for overseeing radiation safety within the facility. The written delegation typically includes:

1. **Official Appointment**: A document that explicitly names the individual designated as the RSO.
2. **Responsibilities**: A clear outline of the RSO's duties and responsibilities related to radiation safety.
3. **Authority**: A statement of the authority granted to the RSO to implement and enforce radiation safety policies and procedures.
4. **Signature**: The document is usually signed by a senior management official or the highest level of authority within the organization, indicating that the designation is officially recognized and supported.

Just as Phoenix Indian Medical Center is required to have a designated Hazardous Material Coordinator to ensure the safe management and compliance of hazardous materials across the facility, it is equally critical that the hospital has a formally delegated Radiation Safety Officer (RSO). The RSO plays a vital role in overseeing the radiation safety program, ensuring compliance with federal, state, and tribal regulations, and safeguarding the health and safety of our patients, staff, and visitors.

The RSO must be officially appointed and possess the necessary qualifications and authority to manage all aspects of radiation safety within the hospital. This includes implementing safety protocols, monitoring radiation exposure, and responding to any radiological incidents. The delegation of an RSO is not only a regulatory requirement but also a fundamental component of our commitment to maintaining a safe and compliant healthcare environment.

Without an officially delegated RSO, PIMC would be at risk of non-compliance with radiation safety regulations, similar to the risks posed by the absence of a Hazardous Material Coordinator. Therefore, it is imperative that this role is clearly defined, formally delegated, and supported by hospital leadership to ensure the highest standards of safety and regulatory compliance are met.

This written delegation is important for compliance with regulatory requirements and must be easily accessible to staff. It serves as an official record that can be presented to regulatory agencies, such as the Nuclear Regulatory Commission (NRC) or state health departments, during inspections or audits.

In summary, having a designated Radiation Safety Officer is a standard requirement for hospitals with Radiology departments to ensure the safe use of radiation and compliance with regulatory standards.

Since these updates involve policies that span across various departments within our organization, these departments must be included in the conversation for awareness, if nothing else. This will ensure that everyone is informed and aligned with the necessary changes.

I see no issue with the transfer of policy ownership to Radiology, provided these critical points are addressed.

Thank you for your attention to this critical matter.

Best regards,

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™
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Phoenix Indian Medical Center
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"The single greatest impediment to error prevention in the medical industry is that we punish people for making mistakes." Dr. Lucian Leape

Legal Disclaimer
The analysis, review, and recommendations provided by the Policy and Procedure Nurse Officer at Phoenix Indian Medical Center (PIMC) are conducted as part of the officer's official responsibilities. These actions are intended to ensure that all policies and procedures within PIMC are thorough, consistent, and compliant with applicable federal, state, and tribal regulations, as well as Indian Health Service (IHS) standards and industry best practices.

The information and recommendations provided by the Policy and Procedure Nurse Officer are intended to support PIMC's commitment to maintaining a safe, compliant, and high-quality healthcare environment. However, this content should not be interpreted as legal advice. PIMC is encouraged to seek guidance from legal counsel or appropriate regulatory authorities to confirm that all policies and procedures fully comply with legal and regulatory requirements.

The Policy and Procedure Nurse Officer does not assume responsibility for the legal adequacy of the policies and procedures reviewed or developed. Phoenix Indian Medical Center retains ultimate responsibility for ensuring that all policies and procedures meet the necessary legal, regulatory, and industry standards.

From: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Date: Thursday, August 29, 2024 at 09:18
To: Numkena, Brian G (IHS/PHX) <Brian.Numkena@ihs.gov>
Cc: Valdisera, Paula (IHS/PHX/PIMC) <Paula.Valdisera@ihs.gov>, Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>, Holiday, Charisse (IHS/PHX) <Charisse.Holiday@ihs.gov>
Subject: Policy ownership

Hi Brian,

We need to transfer ownership of the "Radiation Safety and Monitoring" policy to you or someone in Radiology. LT Valdisera made updates to a draft that is currently in PolicyStat (also attached). As the policy owner, please edit it as needed to manage radiation safety and monitoring to keep everyone safe.

What questions do you have for me? What concerns do you have?

Thanks,

Clint

Very Respectfully,

Clinton D. Pope, MSIE, CPHQ
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